

Danh mục: medication

Type 2 diabetes in adults: management

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NICE guideline

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Complications

1.37 Periodontitis

1.37.1

Advise adults with type 2 diabetes at their annual review that:

they are at higher risk of

periodontitis

if they get periodontitis, managing it can improve their blood glucose control and can reduce their risk of hyperglycaemia.

[2022]

1.37.2

Advise adults with type 2 diabetes to have regular oral health reviews (their oral healthcare or dental team will tell them how often, in line with

NICE's guideline on dental checks: intervals between oral health reviews

[2022]

1.37.3

For guidance for oral healthcare and dental teams on how to provide oral health advice, see

NICE's guideline on oral health promotion

[2022]

1.37.4

For adults with type 2 diabetes who have been diagnosed with periodontitis by an oral healthcare or dental team, offer dental appointments to manage and treat their periodontitis (at a frequency based on their oral health needs).

[2022]

Why the committee made these recommendations

The evidence showed that people with diabetes are at increased risk of periodontitis, and that non-surgical periodontal treatment can improve diabetes control. However, in the committee's experience, people with diabetes are often unaware of this and may not be having regular oral health reviews. To address this, the committee recommended routinely discussing the risk of periodontitis at annual reviews, alongside eye disease and foot problems.

The evidence also showed that periodontal treatment is cost effective for people with type 2 diabetes, assuming improvements in HbA1c are maintained. This was tested with health economic modelling in a range of different scenarios. There were some scenarios where periodontal treatment was not cost effective, but the committee did not

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think these scenarios reflected real-world practice.

Full details of the evidence and the committee's discussion are in

evidence review D: periodontitis

How the recommendations might affect practice

For oral healthcare professionals, the long-term impact of the recommendations is uncertain. The recommendations specify that people should follow existing NICE guidelines on oral health. However, the recommendations may also increase awareness of periodontitis, leading to a possible short-term increase in the number of oral health reviews. Any increase in the number of oral health reviews will potentially impact on services, as NHS dental services already have capacity issues.

A short-term increase in the number of oral health reviews will also lead to a short-term increase in costs. However, there is likely to be a larger long-term reduction in costs from the improvement to oral health and diabetes control.

Oral healthcare and dental teams will need clear advice on what they need to do for people with diabetes. They will need clear care pathways to improve quality of care and service delivery, in line with the

NHS England commissioning standard on dental care for people with diabetes

Many people do not have regular oral health reviews, even if they are eligible for free NHS dental care. People are eligible for free dental care if they are:

pregnant

mothers with babies under 1 year old

on low income benefits, or under 20 and dependent on someone who is receiving low income benefits

having treatment in an NHS hospital by the hospital dentist.

The recommendations may encourage more people with diabetes to have regular oral health reviews. Combined with proactive engagement and enhanced support for people with diabetes, this may broaden access to dental and oral healthcare and help to reduce oral health inequalities.

1.38 Gastroparesis

1.38.1

Think about a diagnosis of gastroparesis in adults with type 2 diabetes who have erratic blood glucose control or unexplained gastric bloating or vomiting, taking into account possible alternative diagnoses.

[2009, amended 2015]

1.38.2

For adults with type 2 diabetes who have vomiting caused by gastroparesis, explain that:

there is no strong evidence that any available antiemetic therapy is effective

some people have had benefit with domperidone, erythromycin or metoclopramide

the strongest evidence for effectiveness is for domperidone, but prescribers must take into account its safety profile, in particular its cardiac risk and potential interactions with other medicines.

[2015]

In February 2026, the use of erythromycin was off label. See

NICE's information on prescribing medicines

1.38.3

To treat vomiting caused by gastroparesis in adults with type 2 diabetes:

consider alternating the use of erythromycin and metoclopramide

consider domperidone only in exceptional circumstances (if domperidone is the only effective treatment) and in accordance with

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MHRA guidance on domperidone

[2015]

In February 2026, the use of erythromycin was off label. See

NICE's information on prescribing medicines

1.38.4

If gastroparesis is suspected, consider referring adults with type 2 diabetes to specialist services if:

the differential diagnosis is in doubt or

the person has persistent or severe vomiting.

[2009]

1.39 Painful diabetic neuropathy

1.39.1

For guidance on managing painful diabetic peripheral neuropathy in adults with type 2 diabetes, see

NICE's guideline on neuropathic pain in adults

[2015]

1.40 Autonomic neuropathy

1.40.1

Think about the possibility of contributory sympathetic nervous system damage in adults with type 2 diabetes who lose the warning signs of hypoglycaemia.

[2009, amended 2015]

1.40.2

Think about the possibility of autonomic neuropathy affecting the gut in adults with type 2 diabetes who have unexplained diarrhoea that happens particularly at night.

[2009, amended 2015]

1.40.3

For adults with type 2 diabetes and autonomic neuropathy who are taking tricyclic antidepressants and antihypertensive medicines, be aware of the increased likelihood of side effects such as orthostatic hypotension. For guidance on safe prescribing of antidepressants and managing withdrawal, see

NICE's guideline on medicines associated with dependence or withdrawal symptoms

[2009]

1.40.4

For adults with type 2 diabetes who have unexplained bladderemptying problems, investigate the possibility of autonomic neuropathy affecting the bladder.

[2009]

1.40.5

In managing autonomic neuropathy symptoms, include specific interventions indicated by the manifestations (for example, for abnormal sweating or nocturnal diarrhoea).

[2009]

1.41 Diabetic foot problems

1.41.1

For guidance on preventing and managing foot problems in adults with type 2 diabetes, see

NICE's guideline on diabetic foot problems

[2015]

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1.42 Erectile dysfunction

1.42.1

As part of the type 2 diabetes annual review, offer to discuss erectile dysfunction (if relevant).

[2015]

1.42.2

Assess, educate and support people with type 2 diabetes who have problematic erectile dysfunction, addressing contributory factors such as cardiovascular disease as well as possible treatment options.

[2015]

1.42.3

Consider a phosphodiesterase5 inhibitor to treat problematic erectile dysfunction in people with type 2 diabetes. Initially choose the medicine with the lowest acquisition cost and take into account any contraindications.

[2015]

1.42.4

After discussion, refer people with type 2 diabetes to a service offering other medical, surgical or psychological management of erectile dysfunction if treatment (including a phosphodiesterase5 inhibitor, as appropriate) has been unsuccessful.

[2015]

1.43 Eye disease

1.43.1

When adults are diagnosed with type 2 diabetes, refer them immediately to the local eye screening service.

[2009, amended 2020]

1.43.2

Encourage adults to attend eye screening, and explain that it will help them to keep their eyes healthy and help to prevent problems with their vision. Explain that the screening service is effective at identifying problems so that they can be treated early.

[2009]

1.43.3

Arrange emergency review by an ophthalmologist for:

sudden loss of vision

rubeosis iridis

pre-retinal or vitreous haemorrhage

retinal detachment.

[2009]

1.43.4

Refer to an ophthalmologist in accordance with the

NHS Diabetic eye screening pathway standards

[2009, amended 2026]

For guidance on managing and monitoring diabetic retinopathy in people under the care of hospital eye services, see NICE's guideline on diabetic retinopathy

1.44 Diagnosing and managing hypertension

The recommendations on diagnosing and managing hypertension have been removed. For recommendations on hypertension in people with type 2 diabetes, see

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NICE's guideline on hypertension in adults

. Diagnosis, treatment and monitoring of hypertension is broadly the same for people with type 2 diabetes as for other people. When a different approach is needed for people with type 2 diabetes, this is specified in the hypertension guideline.

1.45 Antiplatelet therapy

1.45.1

Do not offer antiplatelet therapy (aspirin or clopidogrel) to adults with type 2 diabetes without cardiovascular disease.

[2015]

1.45.2

For guidance on the primary and secondary prevention of cardiovascular disease in adults with type 2 diabetes, see NICE's guidelines on cardiovascular disease

acute coronary syndromes

[2015]